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The Neuropsychology Institute

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NEUROPSYCHOLOGY EVALUATION REPORT

CONFIDENTIAL

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PATIENT NAME: David Nimer
DATE OF EVALUATION: 11/21/2025
DATE OF BIRTH: 05/31/1991 (34 years old)
REFERRED BY: Arizona Supreme Court, Committee on Character and Fitness
NEUROPSYCHOLOGIST: Michael R. Brower, PsyD, CSP, ABPP-CN

DISCUSSION:

Mr. Nimer's medical and psychosocial history has been thoroughly documented in prior psychological and neuropsychological evaluations and will not be reiterated here. For additional details, please refer to those previous reports. During my clinical interview with the examinee (~90 minutes), no significant or concerning discrepancies were noted when compared with information obtained from earlier evaluations.

Regarding information pertinent to the current neuropsychological evaluation, in addition to a history of attention-deficit/hyperactivity disorder (ADHD), Mr. Nimer sustained a significant traumatic brain injury (TBI) in February 2015 when he fell asleep at the wheel and collided with a tree. The exact severity of this TBI is difficult to determine based on the information currently available. He was reportedly conscious at the scene and muttering to the individual who pulled him from the vehicle, suggesting a loss of consciousness of less than 30 minutes (consistent with a mild TBI). His last clear memory before the accident occurred 2–3 hours prior, with his next clear memories emerging 2–3 days later in the hospital, indicating posttraumatic amnesia of 1–7 days (suggestive of moderate severity), though interpretation is complicated by the fact that he was medically induced into a coma. Dr. Kerrigan's 07/04/2018 neuropsychological evaluation referenced medical records documenting a Glasgow Coma Scale (GCS) score of 3, indicative of severe TBI (the records themselves were unavailable for review). Given the intracranial abnormalities noted on neuroimaging, the severity of his TBI is at minimum a complicated mild TBI, though available evidence also supports classification within the moderate-to-severe range; regardless, both complicated mild and moderate-to-severe TBIs are associated with an increased risk of persistent sequelae.

I have no doubt that Mr. Nimer at one point met criteria for mild neurocognitive disorder, as he reported a clear decline from baseline functioning immediately following his TBI. He described motor symptoms (e.g., unsteady gait), somatic symptoms (e.g., head rushing, sleep changes,

fatigue), emotional changes (e.g., decreased frustration tolerance), and cognitive difficulties (e.g., brain fog, reduced attention, poorer organization). In response to his cognitive challenges, he reduced his school workload in order to maintain straight A's. Overall, the available information strongly supports that he experienced a decline from his baseline level of functioning, consistent with a key diagnostic criterion for neurocognitive disorder.

Research indicates that the most significant recovery from a TBI typically occurs within the first 3–6 months post-injury, with milder improvements occurring months to years later, though there is considerable individual variability. In Mr. Nimer's case, he reported substantial and spontaneous improvement during this period, returning to nearly 100% of his baseline functioning within six months. Following his TBI, he completed his bachelor's degree (earning straight A's in his final semester), subsequently obtained a master's degree and Juris Doctorate, performed well on the GRE, and passed the Bar Examination. This pattern of achievement suggests that any lingering sequelae were not sufficiently significant to impede his success.

Additionally, Mr. Nimer completed neuropsychological evaluations in 2016 and 2018, neither of which documented any significant neurocognitive impairment. While both evaluations noted some relative weaknesses (e.g., executive functions), these were mild and interpreted in the context of his intellectual giftedness and ADHD, which may have been potentially complicated by the TBI. Importantly, the reports emphasized that these weaknesses were relative and not indicative of impairment or significant enough to affect his day-to-day functioning. Data from the current neuropsychological evaluation are consistent with prior findings, showing similar performance patterns. Overall, there is no evidence of a consistent pattern of deficits in any cognitive domain.

It is difficult to determine whether Mr. Nimer has achieved full and spontaneous neurocognitive recovery or if lingering sequelae remain that would warrant an ongoing diagnosis of mild neurocognitive disorder, as no baseline evaluation prior to his 2015 TBI is available for comparison. His history of ADHD further complicates the clinical picture, as his relative executive function weaknesses could be attributable solely to ADHD or to a combination of ADHD and his complicated TBI, particularly given the bilateral frontal involvement documented on neuroimaging. Regardless, even if Mr. Nimer has not fully recovered and continues to meet criteria for mild neurocognitive disorder, he demonstrates considerable cognitive reserve. Specifically, he possesses above-average (superior) intellect and advanced educational attainment, and research indicates that individuals with such cognitive reserve are more likely to maintain functional abilities despite the presence of cognitive sequelae.

Of note, Dr. Bradshaw's neuropsychological evaluation referenced a prior assessment from 2011, noting that "a comparison of available data from his 2011 evaluation and the current assessment revealed significant improvements for verbal abstract reasoning, word knowledge, nonverbal deductive reasoning, and mental arithmetic." This 2011 evaluation could potentially serve as a baseline for comparison; however, the report was never received despite a request being sent to Dr. Bradshaw for all medical records. Even though the 2011 evaluation was not available for review, I do not believe its inclusion would significantly alter my conclusions, as Dr. Bradshaw explicitly stated that "no

instances of decline were observed,” suggesting no declines from baseline functioning prior to the 2015 TBI.

Examining Mr. Nimer’s neurocognitive profile more closely, his pattern of strengths and weaknesses supports the conclusion that he is well-positioned to excel as an attorney. Specifically, he demonstrates exceptionally high verbal intellectual abilities (>99th percentile), and his verbal memory performance is also above demographic expectations (above average to exceptionally high)—cognitive domains particularly important for success in legal practice. While he shows relative weaknesses on visual tasks, these domains are not consistently impaired and appear less critical for legal work. Additionally, although his performance on executive function tasks (e.g., working memory, complex attention, cognitive flexibility, hypothesis testing, problem solving, planning) is somewhat variable and represents an area of relative weakness, these limitations occur in the context of his above-average intellect and remain within normal limits for age. Overall, Mr. Nimer’s neurocognitive profile suggests that he possesses the cognitive capacity to succeed as an attorney.

While I am less concerned about Mr. Nimer’s cognitive abilities, I am more concerned about frontally mediated behavioral challenges that could pose potential barriers to his success as an attorney. Specifically, individuals with ADHD and/or TBI involving the frontal lobes often exhibit frontally mediated behavioral difficulties that can impact daily functioning despite intact cognition. During neurocognitive testing in the current evaluation, Mr. Nimer frequently demonstrated such challenges (e.g., disinhibition, tangential, impulse regulation), which affected the testing process (e.g., prolonging the duration of testing). Although Dr. Kerrigan’s evaluation did not document behavioral concerns, Dr. Bradshaw’s report indicated behavioral difficulties endorsed both by Mr. Nimer (e.g., inhibition, task monitoring, planning/organization) and by his mother (e.g., inhibition, set-shifting, emotional control, self-monitoring) on behavioral rating scales. Dr. Bashah’s report also noted notable behavioral observations, including irritability, difficulty redirecting, perseveration, and interpersonal difficulties. It is difficult to determine whether these challenges stem from ADHD, the TBI, personality traits, or a combination of these factors. Regardless, three separate evaluators—Dr. Bradshaw, Dr. Bashah, and myself—have documented potential frontally mediated behavioral challenges.

It is also important to consider the structured nature of the evaluation setting (e.g., quiet room, clearly organized tasks, provided support, clearly elicited responses), which likely facilitates performance at or near his cognitive and behavioral potential. Consequently, neurobehavioral difficulties may be more evident in the complex, unstructured environment of daily life than in this controlled, largely distraction-free testing context—an effect that may be particularly relevant in Mr. Nimer’s case. Overall, frontally mediated behavioral challenges could pose potential barriers to his success as an attorney.

Of note, Dr. Bashah’s psychological evaluation report from 07/18/2025 diagnosed Mr. Nimer with Alcohol Use Disorder (moderate) and Cannabis Use Disorder (mild, in early remission). The examinee reported that he was charged with three DUIs within a 12-month period following his February 2015 TBI. Given the absence of any reported substance-use history prior to the TBI, these DUIs may reflect transient behavioral disturbances (e.g., decreased impulse control) related to the TBI rather than a co-occurring substance use disorder. However, it is important to

emphasize that frontally mediated behavioral challenges associated with both ADHD and TBI represent significant risk factors for the development of a co-occurring substance use disorder.

Regardless, Mr. Nimer emphasized that he completed substance-related treatment following his DUIs and maintained sobriety throughout his two years of probation. Afterwards, he resumed alcohol and marijuana use but self-reported doing so at appropriate and non-problematic levels. He also stated that he has not smoked marijuana in over a year, and his last alcohol consumption was a week prior (a couple of drinks). Importantly, following Dr. Bashah's report, he proactively sought a substance-related evaluation and treatment. He provided a letter from Dr. Manriquez (Medical Director, Addiction Medicine; Banner Health; dated 10/21/2025), which concluded: "...The patient has been assessed by multiple providers and shows no evidence of a substance use disorder per the DSM-V criteria..." Based on the information available at this time, I agree with Dr. Manriquez's conclusions and find no ongoing concerns for a substance use disorder.

CONCLUSIONS:

Overall, Mr. Nimer's neurocognitive profile indicates that he possesses the cognitive capacity to succeed as an attorney, and the current neuropsychological evaluation identified no significant cognitive contraindications that would interfere with his ability to fulfill the duties of this role. If anything, his cognitive strengths—such as exceptionally high verbal intellect—suggest that he has the potential to excel as an attorney

Additionally, I concur with Dr. Manriquez's conclusions that Mr. Nimer does not have a substance use disorder, and I see no need for ongoing substance-related treatment at this time.

However, the primary concern involves frontally mediated behavioral challenges (e.g., disinhibition, poor impulse control, interpersonal difficulties). Regardless of their underlying etiology (e.g., ADHD and/or TBI), these frontally mediated behavioral characteristics may represent potential barriers to Mr. Nimer's success as an attorney.

RECOMMENDATIONS:

Based on this evaluation, it is recommended that should the Attorney Admissions find that Mr. Nimer is eligible for conditional admission, that he be admitted to the practice of law under the following conditions:

1. I recommend that Mr. Nimer continue working with his treating medical provider—ideally one experienced in managing ADHD—to explore treatment options aimed at enhancing frontal-lobe-mediated functions (e.g., medications and/or behavioral interventions). The provider should regularly monitor his progress and stability, and submit documentation to the Arizona Supreme Court indicating whether Mr. Nimer is stable on the most appropriate treatment plan. This approach will ensure that ongoing management of his cognitive and behavioral functioning is formally communicated and assessed. Documentation should be provided at least once every three months for 12 months, or longer if stability has not yet been established.
2. I recommend that Mr. Nimer continue participating in psychotherapy, particularly with a provider experienced in frontally mediated behavioral interventions (e.g., an ADHD

coach). Sessions should initially occur on a weekly basis, with frequency gradually reduced over time (weekly → biweekly → monthly → termination) as determined by the provider based on clinical progress. The provider should offer ongoing feedback and maintain treatment progress notes, which should be submitted to the Arizona Supreme Court to document intervention efforts and monitor improvements in behavior and functional capacity. Upon completion of this intervention, the provider should furnish documentation to the Arizona Supreme Court confirming that all treatment goals have been achieved and that no further behavioral interventions are necessary.

3. I recommend that Mr. Nimer engage with a mentorship program as he transitions into his new professional role as an attorney. The mentor should provide guidance on professional expectations, workplace strategies, time management, and interpersonal interactions, particularly in situations that may challenge frontally mediated behavioral skills (e.g., impulse control, organization, social awareness). Progress notes or periodic summaries from the mentor should be submitted to the Arizona Supreme Court to document ongoing support and monitor Mr. Nimer's adjustment and professional development.
4. Consultation was conducted with Dr. Manriquez on 12/04/2025 via phone. I concur with her assessment that Mr. Nimer does not have a substance use disorder and does not require ongoing substance-related treatment. However, given the examinee's history (e.g., ADHD and TBI), he remains at increased risk for substance abuse. Therefore, he should continue to use substances (e.g., alcohol and marijuana) in a healthy and appropriate manner, as guided by his ongoing treatment provider (e.g., PCP). Annual documentation (e.g., annual physical) should confirm appropriate use and be forwarded to the Arizona Supreme Court for at least the first three years of his career. If stability is demonstrated, further reporting is not required unless new concerns arise. Mr. Nimer must inform any new long-term treating provider of his history, and the new provider must monitor substance use and notify the Arizona Supreme Court if concerns arise.

I make no recommendations regarding whether Mr. Nimer should be admitted to the State Bar of Arizona. Any recommendations regarding the qualifications for admission are beyond the scope of this report. This evaluation and recommendations should be considered only after the Committee on Character and Fitness has reached a decision regarding the recommendation for admission of the Applicant.

Respectfully submitted,



Michael R. Brower, PsyD, CSP, ABPP-CN
Licensed Psychologist, AZ
Board Certified Clinical Neuropsychologist

Please refer to the remainder of this report for additional information regarding consent process, behavioral observations, and test results.

CONSENT PROCESS:

Mr. Nimer was informed by the Arizona Supreme Court, Committee on Character and Fitness, that a psychological evaluation had been recommended, and he voluntarily consented to participate. Prior to the evaluation, he was fully apprised of the purpose of the assessment; the nature of the procedures involved; and the types of tests to be administered, including their general aims and relevance to the referral question. He was advised of the limits of confidentiality applicable to forensic evaluations. Specifically, he was informed that the usual protections of the doctor–patient relationship do not apply in this context, and that information obtained during the evaluation—including his statements, test results, records reviewed, and behavioral observations—may be included in the written report prepared for and submitted to the Arizona Supreme Court, Committee on Character and Fitness. He was also informed of the standard exceptions to confidentiality related to concerns about danger to self or others, as required by state and federal law, and of the evaluator’s record-keeping policies.

With his written and verbal consent, Mr. Nimer affirmed that he understood the nature and purpose of the evaluation, the limits of confidentiality, the potential uses of the information obtained, and his right to ask questions or discontinue participation. He authorized the release of the final report to the Arizona Supreme Court, Committee on Character and Fitness. Mr. Nimer was informed that trainees (e.g., post-doctoral residents) and/or psychometricians may assist with aspects of the neuropsychological evaluation—such as record review, test administration / scoring, and report preparation—under the direct supervision of Dr. Brower. Dr. Brower attests that all conclusions, interpretations, and opinions contained in the final report represent his own professional judgment and constitute his finalized work product.

This evaluation was conducted in accordance with the Ethical Principles of Psychologists and Code of Conduct of the American Psychological Association and the Specialty Guidelines for Forensic Psychology. All data were gathered and interpreted using scientifically grounded and empirically supported methods. Findings are presented in a manner consistent with accepted standards of forensic neuropsychological practice. Throughout the evaluation, the examiner maintained professional neutrality and objectivity, with appropriate consideration of relevant cultural, contextual, and legal factors that may bear on the examinee’s presentation and the referral question.

COLLATERAL SOURCES OF INFORMATION: The examinee provided the following records...

SAT Scores – 2008-2009

A CT Brain without contrast – 02/08/2015

An MRI Brain with and without contrast – 02/08/2015

An MRI of the brain (without and with contrast) – 03/12/2015
University of Colorado transcript – Fall 2013 to Summer 2015
An MRI of the brain (without and with contrast) – 12/02/2015
Neurology progress signed by Satish C. Roa, MD – 12/04/2015
Neuropsychological evaluation report by Major Bradshaw, PhD – 02/02/2016
GRE Scores – 02/08/2018 & 03/14/2018
Neuropsychological evaluation report by J. Michael Kerrigan, PhD – 07/04/2018
Duke University official transcript – Fall 2018 to Spring 2020
Arizona State University official transcript – Fall 2020 to Spring 2023
Arizona Uniform Bar Examination Results – February 2024
Driving records, Arizona Department of Transportation, Motor Vehicle Division – 12/10/2024
Psychological Evaluation by Emily T. Bashah, PsyD – 05/20/2025
Substance Use Disorder Assessment Summary by Maria Manriquez, MD – 10/21/2025
Letter written by J. Michael Kerrigan, PhD – 11/11/2025

In addition, collateral information was obtained via a phone call with Maria Manriquez-Sanchez, MD, on 12/04/2025.

BEHAVIORAL OBSERVATIONS: Mr. Nimer arrived early to the session. He was casually attired, adequately groomed, and demonstrated appropriate hygiene. His gait, posture, and fine motor skills appeared within normal limits, and his eyesight and hearing were adequate for testing. Eye contact was appropriate, and his affect was congruent with his reported “good” mood throughout the evaluation.

Mr. Nimer presented as a social individual. He was a fast talker and frequently engaged in conversation with the administrator. While he often provided lengthy and occasionally tangential responses, his spontaneous speech was otherwise fluent and normal in prosody, volume, rhythm, and tone. He did not demonstrate any speech difficulties (e.g., word-finding issues) and was able to reciprocate conversation while attending to nonverbal cues from the evaluators. Receptive language abilities appeared intact, as he answered questions and followed complex, multi-step directions with minimal-to-no difficulty (i.e., without requiring frequent clarifications or repetitions). Social awareness was sometimes marginal, as the examinee occasionally talked over the examiner and needed to be redirected to the task at hand.

Mr. Nimer’s attention and concentration were variable, with notable impulsivity and disinhibition. During simpler or less engaging tasks (e.g., drawing or questionnaires), he occasionally engaged the evaluator with tangential comments, spoke to himself, and rushed through tasks, sometimes resulting in careless mistakes. On more challenging tasks, he appeared focused and engaged, although some hyperactive behaviors were still observed, such as psychomotor agitation (e.g., tapping his feet or playing with a pencil). He often required redirection to remain on task. Although he tended to work quickly, his overall testing pace was slowed by the previously noted distractions (e.g., engaging the administrator in unrelated conversations). No significant inattentive behaviors were observed (e.g., zoning out). He demonstrated exceptional work stamina, completing 5–6 hours of consecutive neurocognitive testing with minimal breaks—only briefly for the restroom and skipping lunch entirely—showing no signs of fatigue or waning energy.

Mr. Nimer demonstrated fair insight and awareness into his abilities, as he would usually acknowledge his errors and attempt to self-correct. His frustration tolerance was appropriate and he persisted well when challenged. He responded well to encouragement. If anything, he demonstrated a competitive nature, wanting to perform his best on neurocognitive testing.

TESTS ADMINISTERED:

A variety of neuropsychological tests were administered to assess Mr. Nimer's current neurobehavioral functioning. For test security reasons, the specific tests are not listed in this report. An appendix containing the psychometric profile—including the test list, raw scores, standard scores, and percentiles—can be released with written consent from the Arizona Supreme Court.

TEST RESULTS:

NOTE: The following phrases are used to describe performance within the range of percentile ranks indicated in parentheses.

Exceptionally High	(98 or higher)	Low Average	(9-24)
Above Average	(91-97)	Below Average	(2-8)
High Average	(75-90)	Exceptionally Low	(2 or lower)
Average	(25-74)		

Unless otherwise indicated, these descriptors are based upon comparison to age-appropriate statistical norms. When adequate group norms are not available, more general terms reflecting behavioral observations and clinical judgment will be used (e.g., “intact”, “within normal limits” [WNL], “below normal limits” [BNL], “less than adequate,” etc.). Standard scores without identifiers (e.g., z-scores, T-scores, scaled scores) have a mean of 100 and a standard deviation of 15. If present, administrative errors are indicated as AE.

VALIDITY: All stand-alone and embedded performance validity measures were within acceptable limits. Therefore, the test data obtained during the course of this evaluation are therefore believed to represent a valid sampling of Mr. Nimer's current cognitive functioning.

MENTAL STATUS EXAMINATION: Mr. Nimer was administered the Mini Mental State Examination (MMSE), a screening measure for cognitive impairment and dementia. He obtained a MMSE score of 29/30 (28/30 for serial sevens), which is within normal limits for age and education. He lost one point for the county.

INTELLECTUAL ABILITY: Given that Mr. Nimer had already undergone a comprehensive intellectual evaluation with Dr. Bashah on 07/18/2025, this measure was not readministered, as repeating the assessment so soon would likely produce inflated scores due to practice effects. His July 2025 results indicated above-average general intellectual functioning. He demonstrated a pattern of material-specific strengths and weaknesses. Specifically, his verbal intellectual abilities were exceptionally high, whereas his visual-spatial abilities were in the high average

range and his fluid reasoning abilities were average. His auditory working memory and visual processing speed were also average.

ATTENTION/PROCESSING SPEED: Tasks assessing auditory attentional capacity and working memory were consistently within normal limits, with scores typically falling in the average range. However, Mr. Nimer performed significantly better (above average) when the information was more structured and had fewer executive demands.

Visual attentional capacity and working memory, while also within normal limits, were slightly lower than the auditory counterparts. Specifically, Mr. Nimer's scores usually fell in the low average range.

Although all scores were within normal limits, tasks measuring visual attention, scanning, and processing speed showed greater variability, with scores ranging from low average to high average. Tasks involving selective and divided visual attention were also within normal limits (average).

Mr. Nimer was also administered a thorough computerized test that was designed to assess various aspects of attention. Overall, results from this measure indicated that he has no problems with inattentiveness, impulsivity, vigilance, and sustained attention.

LANGUAGE: Performance on tasks involving semantic information and processing was variable. Specifically, confrontation naming fell in the exceptionally low range for age and education, whereas categorical fluency was above average. There was no significant discrepancy between categorical and letter fluency to suggest a semantic processing disruption.

VISUOSPATIAL/CONSTRUCTIONAL ABILITIES: Tasks involving visuospatial processing and construction showed variability. Mr. Nimer performed well when asked to copy a complex figure, demonstrating adequate planning, organization, attention to detail, and appreciation of the figure's gestalt. However, he scored below average on a task requiring the reorganization of puzzle pieces to identify pictures of common objects, which may reflect difficulty with confrontation naming rather than a true visuospatial deficit.

MEMORY: While Mr. Nimer performed better with verbally mediated information, he consistently demonstrated the ability to learn, retain, and recall previously presented information after a delay for both verbal and visual material.

Verbal Memory: Initial acquisition was significantly better (above average) when the information was presented in a structured and organized format, versus when he had to self-impose structure (average). Regardless, he showed that he could improve with repeated learning trials, retain and recall previously learned information after a delay, and benefit from cuing and recognition formats.

Visual Memory: Although Mr. Nimer copied a complex figure accurately, he struggled to recall the information after a brief delay, performing in the exceptionally low range on both immediate and delayed recall trials. However, there was no evidence of additional decay between

immediate and delayed recall. His recognition of the figure's details, however, was within the average range.

Mr. Nimer performed significantly better when recalling simpler but multiple designs. His initial acquisition was in the low average range, but he benefited from repeated learning trials, reaching the high average range by the final trial. He also retained all of the information, performing in the high average range after a delay.

EXECUTIVE FUNCTIONING: While Mr. Nimer's performance was consistently within demographic expectations, some variability and relative weaknesses were observed on tasks involving executive functioning—particularly when considered in the context of his above average intellect. As noted above, he often performed better when information was presented in a structured and organized format, compared to tasks that required him to self-impose structure. Nonetheless, his performance on executive function tasks remained within demographic expectations across domains such as working memory, complex attention, inhibition, cognitive flexibility, hypothesis testing, problem solving, abstraction, deductive reasoning, and initiation/fluency.

MOTOR: Mr. Nimer consistently performed within normal limits on motor tasks, with no consistent evidence for lateralization or psychomotor slowing.

PSYCHOLOGICAL FUNCTIONING: Mr. Nimer's responses on self-report screening measures for mood were consistently within normal limits. It is important to note, however, that self-report measures are less sensitive and not diagnostic; therefore, he also completed a more comprehensive psychodiagnostic assessment. Embedded symptom-validity indicators suggested a tendency to present himself in an overly favorable manner, though this level of under-reporting is not uncommon in fitness-for-duty evaluations and is not sufficient to invalidate the profile. Overall, his clinical profile fell within normal limits, with no evidence of significant psychopathology. At most, and considering potential under-reporting, the results suggest a possible history of alcohol or drug use difficulties. Additionally, there was statistically significant evidence of inflated self-esteem, expansiveness, or grandiosity.

End of Report